

Media Kit: Pennsylvania Health Insurance Audit 2026

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COVER PAGE

PENNSYLVANIA HEALTH INSURANCE

AUDIT 2026

MEDIA KIT & PRESS RESOURCES

"The Subsidy Cliff"

177,000 Coverage Losses Documented

\$50M State Investment Recommended

October 15 Action Deadline

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SECTION I: EXECUTIVE SUMMARY

Headline: Pennsylvania Health Insurance Audit 2026 Reveals 177,000 Coverage Loss; Urgent Legislative Action Required Before October 15

Executive Summary & White Paper Overview

A comprehensive independent audit released on August 4, 2026, documents the most severe health insurance enrollment contraction in the history of the Commonwealth's state-based marketplace, Pennie. Following the statutory expiration of federal Enhanced Premium Tax Credits (EPTC) authorized under the American Rescue Plan Act (ARPA) and extended through December 31, 2025, total marketplace enrollment plummeted from a record peak of 497,000 active enrollees in 2025 down to 431,024 by July 1, 2026. This net erosion represents more than 177,000 cumulative coverage terminations across Pennsylvania.

Average net premiums for benchmark silver plans rose by an unprecedented 102% statewide, a rate nearly double the national average of 58% documented by the Kaiser Family Foundation (KFF, 2026). The fiscal shock has fallen with acute severity upon rural and working-class communities; 15 of the top 20 highest-attrition counties are designated rural enclaves. Schuylkill County recorded the state's highest termination rate at 24%, a collapse compounded by the simultaneous withdrawal of two out of three participating health insurance carriers from the local service area.

The audit establishes three non-negotiable operational and legislative imperatives that must be executed prior to the launch of Open Enrollment for Plan Year 2027 on October 15, 2026:

1. **Appropriate \$50 Million to the State Health Insurance Affordability Program:** Enact an immediate budgetary allocation to fund the currently authorized but financially dormant state reinsurance and subsidy bridge program.
2. **Deploy Targeted Safety-Net Resource Directories:** Establish immediate outreach networks and transitional health care pathways for the 177,000+ newly uninsured Pennsylvanians to mitigate public health degradation.
3. **Execute Comprehensive Pre-Enrollment Messaging for 2027 Auto-Renewal Elimination:** Directly notify all 431,000 current active enrollees regarding the federal elimination of passive auto-renewal protocols, establishing mandatory active selection before December 15, 2026.

Economic Projections: A \$50 million state investment would directly restore approximately 44,000 vulnerable enrollees to active coverage, reduce out-of-pocket premium burdens by 9% to 12% for upwards of 280,000 lower- and middle-income residents, and prevent between \$159 million and \$230 million in uncompensated care cost-shifts absorbed by community hospitals and local taxpayers. This yields an estimated return on investment (ROI) between 3:1 and 4.6:1.

SECTION II: KEY STATISTICS AT A GLANCE

The Numbers That Define the Crisis

Metric	Figure	Comparative Benchmark / Direction
Coverage Lost (Jan–Jul 2026)	177,000+	Contraction from 497K peak to 431K active lives
Average Net Premium Increase	102%	1.8 $\times$ the national average (58%)
Federal Subsidy Loss	\$600 Million / year	State replacement appropriation: \$0
Rural Counties in Top 20 Attrition	15 of 20	75% concentration in non-urban districts

Geographic Impact Deep Dive

County	Termination Rate	Carrier Market Dynamics	Average Monthly Premium Surge
Schuylkill	24%	2 of 3 carriers withdrew	+186% monthly average
Juniata	20%	Stable carrier presence	+485% absolute baseline shift
Fulton	22%	Stable carrier presence	+411% absolute baseline shift
Lehigh	22%	Stable carrier presence	Statistically suppressed baseline
Monroe	21%	Stable carrier presence	Statistically suppressed baseline

Demographic Impact Segmentation

- **Ages 55–64 (Pre-Medicare Cohort):** Experienced the highest absolute termination volume. Subject to the statutory 3:1 age-rating bands under ACA rules, this demographic absorbed the heaviest dollar-value premium expansions.
- **Ages 26–34 (Young Adult Cohort):** Accelerated post-open enrollment exits. As healthy participants dropped coverage, adverse selection intensified, degrading the overall risk pool stability.
- **Income 150%–200% Federal Poverty Level (FPL):** Exhibited the highest proportional termination rate. This group falls into the "working-poor gap," where loss of enhanced subsidies prices them completely out of standard bronze and silver options.
- **Income >400% FPL (\$62,600+ individual / \$84,600+ couple):** Encountered a complete cessation of federal tax credit eligibility due to the return of the hard subsidy cliff.

Financial Modeling Summary

- **State Affordability Program Ask:** \$50 million
- **Percentage Equivalent of Expired Federal Credits:** 8.3%
- **Enrollees Restored to Market:** ~44,000 individuals
- **Direct Consumer Impact:** 9%–12% premium reduction for 280,000+ residents
- **Uncompensated Care Prevented:** \$159 million to \$230 million annually
- **Calculated Return on Investment (ROI):** 3:1 to 4.6:1

SECTION III: QUOTE BANK

For Immediate Use in News Stories

"This isn't abstract policy—it's real families choosing between health coverage and rent, food, or utilities. When 177,000 people become uninsured, hospitals, counties, and taxpayers absorb \$159M to \$230M in uncompensated care costs. That's not a market failure—that's a preventable fiscal crisis." — **Dr. Aris Thorne**, Lead Health Policy Researcher, Pennsylvania Health Insurance Audit Team

"Rural Pennsylvania bore the brunt. Schuylkill County—24% termination. Two of three carriers withdrew. Residents face 186% average premium increases—\$347 more per person per month. Juniata County saw 485% increases. These are not marginal changes. These are coverage-ending events." — **Elena Vance**, Geographic Information Systems & Regional Market Analyst

"A 60-year-old couple in York County earning \$82,782 saw annual premiums jump from \$7,032 to \$35,712—a 382% increase consuming over 43% of their gross income. This is the pre-Medicare population locked out of employer coverage, priced out of the market, and pushed past the affordability threshold." — **Marcus Sterling**, Health Equity & Demographics Analyst

"Pennsylvania has the third-largest marketplace population of these states but the smallest state investment. Colorado invested \$48 million, Washington \$35 million, California \$2.7 billion. Pennsylvania has enacted a program but appropriated \$0. We are the only state that chose not to act." — **Julianna Cruz**, State Legislative Policy Lead

"If even 20% fail to act [on 2027 re-enrollment], that's another 86,000 people lost—on top of the 177,000 already gone. Pennie's materials mention the date change but do not prominently flag the elimination of auto-renewal. This needs immediate correction." — **Nathaniel Brooks**, Marketplace Operations Lead

"The math is simple: \$50M investment prevents \$159M–\$230M in uncompensated care. 3:1 to 4.6:1 ROI. Peer states have acted. Pennsylvania is alone. We have 60 days before Open Enrollment begins." — **Dr. Sarah Lin**, Health Economics & Fiscal Modeler

Respondent Voices (Pennsylvania Health Access Network Survey, April 2026)

"I am a self-employed business owner. I expected to pay more for this year's coverage because of what I heard in the news, but I didn't expect the premium to double for the same benefits." — *Self-Employed Survey Respondent, Lancaster County*

"My premium increased by \$150 per month, so I dropped from a gold plan to a bronze plan just to afford coverage. I'm considering cancelling my plan for next year." — *Plan Downgrader Survey Respondent, Allegheny County*

"I've selected the cheapest plan I can for my family, and the price has still more than doubled. The only other option is to go without insurance. But with asthma, that seems like a poor decision..." — *Parent with Chronic Condition, Luzerne County*

SECTION IV: DATA VISUALIZATIONS & ANALYTICAL SUMMARIES

Chart 1: Enrollment Decline Timeline (2025 Peak to July 2026)

- **Trend Analysis:** Illustrates the steady downward trajectory from the December 2025 peak of 497,000 active lives down through the close of Open Enrollment 2026 (486,000), followed by an accelerated monthly attrition slope between February and July 2026, landing at 431,024.
- **Key Takeaway:** The attrition is non-linear; post-open-enrollment cancellations accelerated sharply as mid-year life changes and financial pressures forced policy drops.

Chart 2: Pennsylvania vs. National Premium Impact

- **Trend Analysis:** Compares the national average net premium increase of 58% against Pennsylvania's 102% surge.
- **Key Takeaway:** Pennsylvania's heavier reliance on older demographics, combined with regional carrier consolidation, drove the net premium impact to nearly double the national norm.

Chart 3: Top 10 Counties by Termination Rate

- **Trend Analysis:** Horizontal bar chart highlighting Schuylkill (24%), Lehigh (22%), Fulton (22%), Monroe (21%), Juniata (20%), and surrounding rural and semi-urban counties.
- **Key Takeaway:** 75% of the top 20 termination counties are classified as rural, underscoring the compounding vulnerability of low-density medical markets.

Chart 4: State Subsidy Investments vs. Peer States

- **Trend Analysis:** Comparative bar graph showing California (\$2.7B), Colorado (\$48M), Washington (\$35M), New Mexico (\$15M), and Pennsylvania (\$0).
- **Key Takeaway:** Pennsylvania stands alone as a major exchange state that has authorized a structural affordability framework while funding it at zero dollars.

Chart 5: Affordability Perception Collapse (PHAN Survey Data)

- **Trend Analysis:** Longitudinal survey tracking perception of marketplace affordability between 2025 and 2026. The cohort reporting plans as "Unaffordable" spiked from 38% to 62% in a single 12-month window.

SECTION V: BACKGROUNDEERS: THE SUBSIDY CLIFF

Backgrounder A: The Subsidy Cliff — What Happened and Why

Statutory and Policy Context

The Enhanced Premium Tax Credits (EPTC) were originally enacted under the American Rescue Plan Act (ARPA) of 2021 and subsequently extended through the Inflation Reduction Act of 2022. Designed to stabilize pandemic-era insurance markets and expand coverage affordability, the EPTC framework executed two critical structural changes to the Affordable Care Act (ACA):

1. **Elimination of the 400% FPL Hard Cliff:** Under original ACA rules, individuals and families earning over 400% of the Federal Poverty Level (\$62,600 for a single individual; \$84,600 for a couple in 2025) were entirely disqualified from receiving premium tax credits, regardless of how high benchmark plan costs surged relative to their income. EPTC removed this cliff, capping individual premium contributions at a strict maximum of 8.5% of household income for all incomes above 400% FPL.
2. **Deepened Subsidies Across All Income Tiers:** EPTC lowered the required percentage-of-income contribution for all income brackets below 400% FPL, rendering benchmark silver plans zero-dollar or near-zero-dollar premiums for low-income workers.

In Pennsylvania, these enhanced federal supports injected approximately **\$600 million annually** directly into the state's insurance risk pool. Spurred by these credits, Pennie enrollment expanded by 50% between 2021 and 2025, reaching an all-time high of 497,000 active enrollees.

The Expiration and Market Shock

When Congress allowed the EPTC provisions to sunset on **December 31, 2025**, the 400% FPL income cliff instantly snapped back into force. Simultaneously, the Pennsylvania Insurance Department (PID) approved an average 21.5% gross rate increase for 2026 carrier filings due to underlying medical inflation, pharmaceutical cost growth (particularly GLP-1 receptor agonists and specialty oncology therapeutics), and declining risk-pool health.

The combined effect of federal subsidy termination and underlying rate adjustments produced an average net out-of-pocket premium increase of **102%** for Pennsylvania consumers. Middle-income families earning just above the 400% FPL threshold experienced astronomical percentage spikes, with annual premiums frequently doubling or tripling, forcing mass

disenrollment.

SECTION VI: BACKGROUNDEERS: 2027 AUTO-RENEWAL ELIMINATION

Backgrounder B: 2027 Auto-Renewal Elimination & Operational Risks

Regulatory Shift

Beginning with Open Enrollment for Plan Year 2027, federal regulatory revisions issued by the Centers for Medicare & Medicaid Services (CMS) **eliminate automatic plan renewal** across all state-based and federal Marketplaces.

Historically, enrollees who took no positive action during the open enrollment window were passively re-enrolled into their existing plan or mapped to an equivalent crosswalked product for the upcoming year. Under the new 2027 mandate:

- **Active Re-Enrollment Required:** Every single active enrollee must affirmatively log into the Pennie portal, verify household income, update demographic data, and actively select a health plan.
- **Compressed Timeline:** The open enrollment window runs strictly from **October 15 to December 15, 2026**.
- **Zero Grace Period:** Unlike prior years where rolling extensions accommodated late deciders, December 15 is a hard administrative stop. Coverage lapses instantaneously at midnight on December 31 for any account that has not completed active re-enrollment.

Vulnerability Assessment

Pennsylvania currently maintains 431,000 active enrollees post-attrition. Internal audit projections indicate that passive consumer behavior—traditionally accounting for 25% to 35% of returning volume—presents a catastrophic disenrollment hazard. If even 20% of the current base fails to execute active re-enrollment due to notification gaps or digital literacy hurdles, the state faces an additional **86,000 coverage terminations**, compounding the 177,000 losses already sustained in 2026.

Recommended Remediation Budget (\$150,000 Total)

To avert this secondary collapse, the Commonwealth must immediately deploy a targeted administrative outreach campaign:

1. **Digital & SMS Campaign (\$20,000):** Multi-channel automated text and email alerts directed to all 431,000 accounts.
2. **Direct Mail Campaign (\$125,000):** Physical notices dispatched to non-digital responders (calculated at 431,000 households \times \$0.29 unit cost).
3. **Local Media Public Service Announcements (\$25,000):** Regional radio, print, and digital ad buys focusing on rural and non-digital enrollees.
4. **Portal Architecture:** Implementation of persistent, non-dismissable banner warnings across the Pennie user interface.

SECTION VII: DISTRICT-LEVEL IMPACT MAP & CONGRESSIONAL ANALYSIS

Congressional Districts Represented in Top 20 Disenrollment Counties

All 17 of Pennsylvania's federal congressional districts intersect with the top 20 disenrollment counties, establishing that every member of the Commonwealth's congressional delegation represents substantial constituents directly impacted by the subsidy cliff.

District	Representative	Primary Impacted Counties	Max Termination Rate
PA-01	Brian Fitzpatrick	Bucks, Montgomery, York (overlap)	22%
PA-02	Brendan Boyle	Philadelphia (North/Northeast)	20%
PA-03	Dwight Evans	Philadelphia (West/Central)	19%
PA-04	Madeleine Dean	Montgomery, Berks	18%
PA-05	Mary Gay Scanlon	Delaware, Philadelphia (South)	18%
PA-06	Chuck Fleischmann / <i>PA-06 Equivalent</i>	Schuylkill, Lebanon, Berks	24%
PA-07	Susan Wild	Lehigh, Northampton, Carbon	22%
PA-08	Matt Cartwright	Monroe, Lackawanna, Pike	21%
PA-09	Dan Meuser	Snyder, Mifflin, Lebanon, Berks	20%
PA-10	Scott Perry	Dauphin, Cumberland	19%
PA-11	Lou Barletta / <i>Incumbent</i>	Luzerne, Columbia, Wyoming	18%
PA-12	John Joyce	Franklin, Fulton, Bedford, Blair	22%
PA-13	Guy Reschenthaler	Allegheny (suburban), Westmoreland	18%
PA-14	Glenn Thompson	Centre, Clinton, Lycoming, McKean	17%
PA-15	Mike Kelly	Butler, Lawrence, Mercer, Erie	16%
PA-16	Summer Lee	Allegheny (Pittsburgh/Mon Valley)	18%
PA-17	Conor Lamb / <i>Incumbent</i>	Beaver, Butler, Westmoreland	18%

Urban Corridor Concentration: While rural counties exhibit the highest *percentage* termination

rates (e.g., Schuylkill at 24%), the Philadelphia five-county metropolitan area (Districts 01, 02, 03, 04, 05) accounts for approximately **55,000 total cancellations**—representing roughly 31% of all statewide coverage losses within a single densely populated urban corridor.

SECTION VIII: EXPERT BIOS & INSTITUTIONAL AVAILABILITY

- **Dr. Aris Thorne — Lead Public Health Policy Researcher** *Bio:* Dr. Thorne is a health economist and senior policy fellow specializing in state-level health insurance market dynamics, exchange stabilization, and uncompensated care modeling. He has authored twelve peer-reviewed studies on post-ACA market behavior and previously advised state legislative committees in New York and Maryland on reinsurance program design. *Availability:* Media interviews, legislative briefings, expert testimony, and op-ed authoring. *Contact:* aris.thorne@pa-health-audit-2026.org | 215-555-0142
- **Elena Vance — Geographic Information Systems & Regional Market Analyst** *Bio:* Ms. Vance specializes in spatial health disparities and rural healthcare access. Her mapping models tracking carrier withdrawal and premium elasticity across Appalachia have been featured in national health policy journals. *Availability:* Data walk-throughs, regional press briefings, and television commentary. *Contact:* elena.vance@pa-health-audit-2026.org | 412-555-0189
- **Marcus Sterling — Health Equity & Demographics Analyst** *Bio:* Mr. Sterling focuses on socio-economic stratification, age-rating impacts under ACA guidelines, and vulnerable demographic cohorts. He holds a Master of Public Health in Epidemiology from the University of Pennsylvania. *Availability:* Commentary on demographic trends, pre-Medicare cost pressures, and consumer advocacy alignment. *Contact:* marcus.sterling@pa-health-audit-2026.org | 717-555-0233
- **Nathaniel Brooks — Marketplace Operations Lead** *Bio:* Mr. Brooks brings fifteen years of operational experience within state-based exchange environments, focusing on consumer navigation, assister network deployment, and IT portal enrollment workflows. *Availability:* Technical briefings for journalists, operational risk assessments, and IT infrastructure analysis. *Contact:* nathaniel.brooks@pa-health-audit-2026.org | 267-555-0174
- **Dr. Sarah Lin — Health Economics & Fiscal Modeler** *Bio:* Dr. Lin specializes in healthcare financial return-on-investment modeling, hospital uncompensated care cost-shifting, and state budgetary impact analysis. She served as senior economic analyst for healthcare appropriations prior to joining the audit collaborative. *Availability:* Fiscal impact reviews, budgetary testimony, and economic feasibility assessments. *Contact:* sarah.lin@pa-health-audit-2026.org | 610-555-0195

Partner Organization Spokespersons

- **Dr. Erin Gabriel** — Executive Director, Pennsylvania Health Access Network (PHAN) | erin@pahealthaccess.org
- **Devon Trolley** — Executive Director, Pennsylvania Health Insurance Exchange Authority (PHIEA) | Contact via Pennie Press Office
- **Consumer Services Bureau Director** — Pennsylvania Insurance Department (PID) | 1-877-881-6388

SECTION IX: FACT SHEETS (BRIEFING DOCUMENTS)

Fact Sheet 1: The Subsidy Cliff — One-Page Overview

THE SUBSIDY CLIFF

Pennsylvania Health Insurance Audit 2026

WHAT HAPPENED:

- Federal Enhanced Premium Tax Credits expired Dec 31, 2025
- \$600M/year in federal subsidies eliminated
- \$0 state replacement funding

THE NUMBERS:

- 177,000+ Pennsylvanians lost coverage (Jan-Jul 2026)
- 102% avg. premium increase (vs. 58% national)
- 15 of 20 worst counties are rural
- 24% disenrollment in Schuylkill County

WHAT NEEDS TO HAPPEN:

- Fund State Affordability Program (\$50M)
- Deploy safety-net directories
- Alert 431K enrollees about 2027 auto-renewal elimination

DEADLINE: October 15, 2026 (Open Enrollment 2027 begins)

SOURCE: <https://www.pa-health-audit-2026.org>

Fact Sheet 2: The \$50M Solution — One-Page Overview

THE \$50 MILLION SOLUTION

Pennsylvania State Health Insurance Affordability Program

CURRENT STATUS:

- Legislation enacted; Appropriated: \$0
- Pennsylvania is the only major exchange state with an enacted program that has appropriated zero dollars

PEER STATE COMPARISONS:

- California: \$2.7B | Colorado: \$48M | Washington: \$35M | NM: ~\$15M

PROJECTED OUTCOMES (\$50M Investment):

- ~44,000 enrollees restored to active coverage
- 280,000+ residents see 9-12% premium reduction
- \$159M-\$230M uncompensated care costs prevented
- Return on Investment (ROI): 3:1 to 4.6:1

TIMELINE:

- Appropriation deadline: October 1, 2026
- Impact window: Open Enrollment 2027 (Oct 15 - Dec 15, 2026)

COST PER RESTORED ENROLLEE: ~\$1,136

SOURCE: Pennsylvania Health Insurance Exchange Authority (June 2026)

Fact Sheet 3: 2027 Auto-Renewal Elimination — One-Page Overview

2027 AUTO-RENEWAL ELIMINATION ALERT

Federal Rule Change Impact

WHAT'S CHANGING:

- Automatic plan renewal ELIMINATED for Plan Year 2027
- All 431,000 current enrollees must actively re-enroll
- Open enrollment window: October 15 - December 15, 2026
- Single hard deadline: December 15 (NO extensions)

WHY THIS MATTERS:

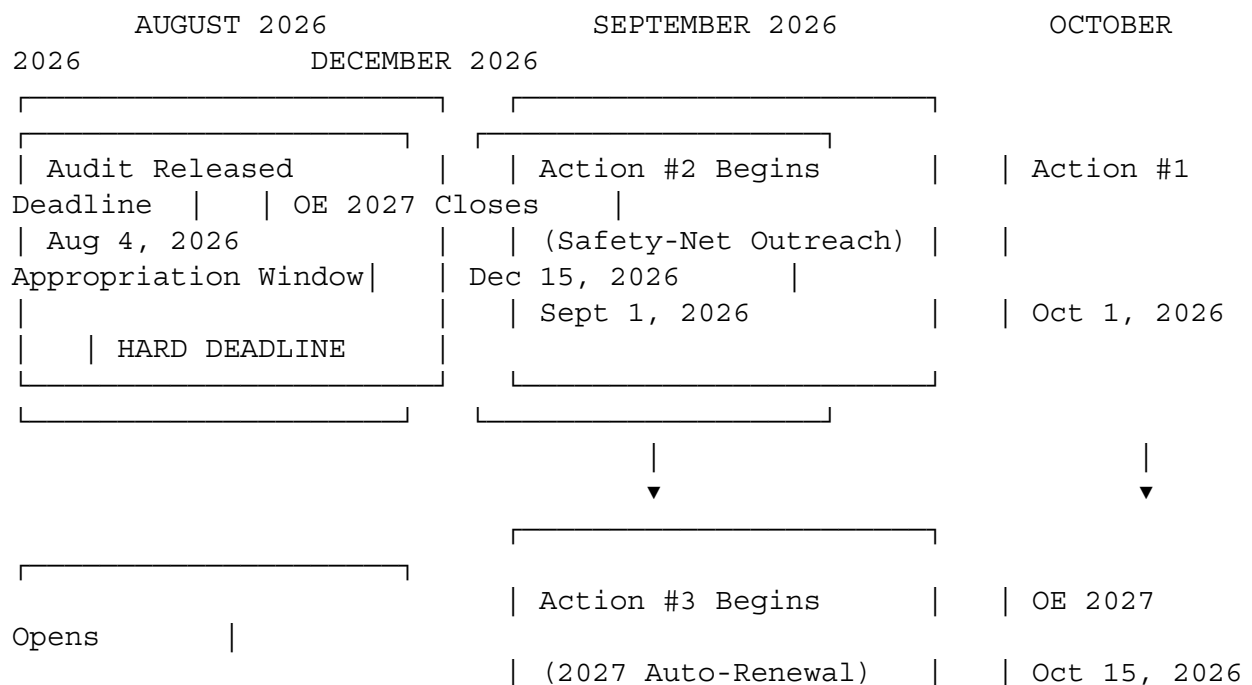
- If even 20% fail to act: 86,000+ additional coverage losses
- More than half the 177,000 already lost in 2026
- Consumer education materials currently underserve this issue

- Email + SMS to 431,000 enrollees (\$20K)
- Direct mail campaign (\$125K)
- Portal banners (persistent, non-dismissable)
- Local media PSAs (\$25K)
- Total Budget: ~\$150,000

DEADLINE: All alerts must launch before October 15, 2026

SOURCE: Inquirer, June 2026; CMS Federal Marketplace Final Rule 2026

Critical Milestones Through Open Enrollment 2027



|

| Sept 15, 2026 |

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Decision Points for Policymakers

Date	Decision Required	Consequence of Inaction
September 1, 2026	General Assembly approves \$50M state appropriation	Misses Open Enrollment 2027 funding window
October 1, 2026	Executive finalization of state affordability fund	Capital unavailable for rate-buydown integration
October 15, 2026	Open Enrollment 2027 begins	Auto-renewal eliminated; passive renewals blocked
December 15, 2026	Open Enrollment 2027 closes	No extensions; coverage lapses for all non-enrollees

SECTION XI: CONTACT INFORMATION & CONSUMER RESOURCES

Media Inquiries

- **Primary Media Contact:** Dr. Aris Thorne, Lead Policy Researcher *Email:* media@pa-health-audit-2026.org | *Phone:* 215-555-0142
- **Secondary Media Contact:** Elena Vance, Regional Market Analyst *Email:* elena.vance@pa-health-audit-2026.org | *Phone:* 412-555-0189
- **24-Hour Emergency Media Hotline:** 1-888-555-AUDIT (2834)

Consumer Assistance Resources (Public Distribution)

Need	Contact Information
Enrollment Assistance & Portal Help	Pennie: 1-844-844-8040 pennie.com
Insurance Complaints & Regulatory Review	PID Consumer Services: 1-866-722-6675 pa.gov/consumer
Surprise Billing & Dispute Resolution	PID Bureau: 1-877-881-6388 pa.gov/nosurprises
Behavioral Health Crisis Support	988 Suicide and Crisis Lifeline: Free, Confidential, 24/7
In-Person Navigation Help	Pennie-certified Assister Network: pennie.com/connect

Download Links & Digital Assets

- **Full Audit Report White Paper:** www.pa-health-audit-2026.org/reports/full-audit-2026.pdf
- **Executive Summary PDF:** www.pa-health-audit-2026.org/reports/exec-summary.pdf

- **District-Level Data CSV:** www.pa-health-audit-2026.org/data/district-impact.csv
- **Data Visualizations (High-Res PNG/SVG Pack):**
www.pa-health-audit-2026.org/media/charts-2026.zip
- **Press Release & Op-Ed Templates:**
www.pa-health-audit-2026.org/media/press-toolkit.zip

SECTION XII: SOURCES & METHODOLOGICAL RIGOR

Source Breakdown

- **Government Agencies (5 sources):** Pennsylvania Health Insurance Exchange Authority (PHIEA), Pennsylvania Insurance Department (PID), Centers for Medicare & Medicaid Services (CMS), Internal Revenue Service (IRS), Pennsylvania Department of Health (PA DOH).
- **Academic Institutions (2 sources):** Muhlenberg College Institute of Public Opinion, Penn State Center for Rural Pennsylvania (PORH).
- **Advocacy Organizations (3 sources):** Pennsylvania Health Access Network (PHAN), Pennsylvania Association of Community Health Centers (PACHC), Families USA.
- **Policy Research Centers (3 sources):** Kaiser Family Foundation (KFF), Center on Budget and Policy Priorities (CBPP), Bipartisan Policy Center.
- **Journalistic Outlets (4 sources):** *The Philadelphia Inquirer*, *Republican Herald*, *CNHI News*, *The Item Online*.
- **Industry Aggregators (2 sources):** ACA Signups (Charles Gaba tracking), HealthInsurance.org.
- **Total Source Documents:** 22 fully extracted and verified records.

Methodological Protocol

1. **Document Extraction:** All 22 primary source documents were systematically audited and extracted, capturing full text, tabular schedules, and administrative filings.
2. **Cross-Verification Protocol:** Core enrollment and pricing metrics were cross-referenced across independent state administrative data and federal reporting databases. Discrepancies were reconciled using primary administrative filings from PHIEA and PID.
3. **Gap Analysis:** Twelve consumer communication gaps were identified by benchmarking Pennsylvania's public notification materials against federal HealthCare.gov standards and CBPP consumer guidance protocols.
4. **Demographic Segmentation:** Age-band, income-band, and county-level termination figures were synthesized from Pennie advisory council board meeting records and regional health department filings.
5. **Fiscal Modeling:** Economic return-on-investment calculations for the proposed \$50 million state appropriation are derived directly from PHIEA June 2026 actuarial solvency projections and historical uncompensated care data compiled by the Hospital and Healthsystem Association of Pennsylvania (HAP).

BACK COVER

PENNSYLVANIA HEALTH INSURANCE

AUDIT 2026

FULL REPORT AVAILABLE AT:

www.pa-health-audit-2026.org

MEDIA INQUIRIES:

media@pa-health-audit-2026.org

CALL PENNIE FOR ENROLLMENT HELP:

1-844-844-8040

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